Chakravarthy, director of internal medicine at Fortis Hospital, Bengaluru. When shingles erupts, it starts to affect the skin around these nerves. "In some people, it can erupt near the ear and cause infection, leading to hearing and balance issues. In others, it can happen inside the mouth, making it painful to eat. In some, it can happen in and around the eyes, eyelids, forehead and even the tip of the nose."

The blisters need to be kept clean to avoid other bacterial infections, warns Dr D.M. Mahajan, a dermatologist with Apollo Hospitals, Delhi. "You need to consult a doctor immediately to know how to look after yourself," he adds, "as it depends from case to case." It is also important to get quarantined, as the virus can be transmitted to others, especially to those who have not had chickenpox, through direct contact with the open sores. Like all viral infections, shingles is self-limiting and the blisters usually dry up and crust over within seven to 10 days. For many, though, this is not the end of the disease.

The third phase plays out in people with low immunity, three to four months after the rash fades. "It is an excruciating pain that can last for six months to one year," says Dr Budhiraja. "This happens because damaged nerve fibres send exaggerated messages of pain from the skin to your brain." In fact, pain was the most distressing symptom for 79 per cent of the patients surveyed in the API-Ipsos study. "It can make daily activities such as walking, bathing and dressing difficult for ageing adults," notes Dr Hegde, "and increase their dependence on caregivers." In some patients, shingles has also been known to cause vision loss, facial paralysis, bacterial skin infections and inflammation of the brain.

Why does the virus suddenly reactivate after having stayed dormant for years? "Lowered immunity due to age, other medications or illnesses," says Mumbai-based infectious disease expert Dr Om Shrivastava, "can reduce the body's ability to fight the virus." During the Covid pandemic, for instance, many doctors in India reported an increase in cases of shingles as the administration of steroids or the disease itself impacted immunity. In the study cited earlier, 66 per cent of the patients surveyed reported chronic conditions such as diabetes and hypertension, indicating a possible link between the two. Other possible causes are diseases like HIV infection and cancer or treatments like radiation or medications to prevent the rejection of transplanted organs—all of which lower immunity and increase the risk of reactivating the shingles virus.

It is to reduce this risk that doctors strongly advocate that children get vaccinated against chickenpox. And in case you have already had chickenpox, to get vaccinated against shingles as you age. Zostavax, a live vaccine for shingles, has been around since 2006, but its production was discontinued in the US in 2020. Shingrix, on the other hand, is the first non-live vaccine—considered to be safer as it uses a killed version of the virus instead of a weakened form—that was approved in the US in 2017 and in the European Union a year later, and is today being used globally. An extension study, titled 'Zoster-049', released last year shows that Shingrix can provide at least a decade of protection against shingles after initial vaccination. Per the CDC data, among people with a healthy immune system, Shingrix was 97 per cent effective in preventing shingles in the 50-69 age group, and 91 per cent in those aged above 70. When it came to preventing PHN, the efficacy stood at 91 per cent and 89 per cent, respectively. Even among adults with a weakened immune system, Shingrix was 68 per cent to 91 per cent effective, depending on the underlying condition.

"The vaccine doesn't guarantee you won't get shingles, but it greatly reduces the risk and possible complications," says Dr Budhiraja. "Even if you have had an episode of shingles, you should still take the vaccine, so the risk of getting it again with severity is much lower." But the adoption of adult vaccination remains a hurdle in India, not just for shingles but for other preventable diseases like cervical cancer and flu as well. Another API-Ipsos study conducted this year among 1,950 people aged above 50 across 16 cities found that although 71 per cent of them were aware of adult vaccination, a dismal 16 per cent had taken any jab. The reasons range from misconceptions like multiple doses could make them dependent on vaccines (50 per cent respondents) to simply not enquiring about adult vaccination as they believe that if they need it, their doctors would recommend it (69 per cent).

"There's a need to tackle a lot of misconceptions about adult vaccines to improve adoption," says Dr Sharad Kumar Agarwal, president, Indian Medical Association. "Vaccines not only save lives, but also improve the quality of life." If a disease is preventable, there is no reason to choose pain and suffering over your health and wellbeing. ■

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–DR SANDEEP BUDHIRAJA

Medical Director, Max Healthcare

